Patient Name	: Mrs. SRIPRIYA	Collected	: 08/Feb/2026 09:04AM
Age/Gender	: 42 Y 0 M 0 D /F	Received	: 08/Feb/2026 11:07AM
UHID/MR No	: DRUL.0000019139	Reported	: 08/Feb/2026 01:56PM
Visit ID	: DRULOPV19672	Status	: Final Report
Ref Doctor	: DR LEKSHMI NARENDRAN	Client Name	: PUP MED COUNT DIAGNOSTICS
IP/OP NO	:	Center location	: Kathriguppe,Bangalore

DEPARTMENT OF HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
COMPLETE BLOOD COUNT (CBC) , *WHOLE BLOOD EDTA*				
HAEMOGLOBIN	12.7	g/dL	12.5-15	Spectrophotometer
PCV	38.50	%	36-46	Electronic pulse & Calculation
RBC COUNT	**5.05**	Million/cu.mm	3.8-4.8	Electrical Impedence
MCV	**76.2**	fL	83-101	Calculated
MCH	**25.2**	pg	27-32	Calculated
MCHC	33.1	g/dL	31.5-34.5	Calculated
R.D.W	13.5	%	11.6-14	Calculated
TOTAL LEUCOCYTE COUNT (TLC)	**10,320**	cells/cu.mm	4000-10000	Electrical Impedance
DIFFERENTIAL LEUCOCYTIC COUNT (DLC)				
NEUTROPHILS	78	%	40-80	Flow cytometry
LYMPHOCYTES	**17**	%	20-40	Flow cytometry
EOSINOPHILS	2	%	1-6	Flow cytometry
MONOCYTES	3	%	2-10	Flow cytometry
BASOPHILS	0	%	0-2	Flow cytometry
CORRECTED TLC	10,320	Cells/cu.mm		Calculated
ABSOLUTE LEUCOCYTE COUNT				
NEUTROPHILS	**8049.6**	Cells/cu.mm	2000-7000	Calculated
LYMPHOCYTES	1754.4	Cells/cu.mm	1000-3000	Calculated
EOSINOPHILS	206.4	Cells/cu.mm	20-500	Calculated
MONOCYTES	309.6	Cells/cu.mm	200-1000	Calculated
Neutrophil lymphocyte ratio (NLR)	**4.59**		0.78- 3.53	Calculated
PLATELET COUNT	385000	cells/cu.mm	150000-410000	Electrical impedence
MPV	8.9	Fl	8.1-13.9	Calculated

Dr.Mahalakshmi S
M.B.B.S,M.D(Pathology)
Consultant Pathologist

SIN No:704638167E

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DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval	Method
GLUCOSE, FASTING , *NAF PLASMA*	**118**	mg/dL	70-100	Hexokinase

Comment:

As per American Diabetes Guidelines, 2023

Fasting Glucose Values in mg/dL	Interpretation
70-100 mg/dL	Normal
100-125 mg/dL	Prediabetes
≥126 mg/dL	Diabetes
<70 mg/dL	Hypoglycemia

Note:

1.The diagnosis of Diabetes requires a fasting plasma glucose of > or = 126 mg/dL and/or a random / 2 hr post glucose value of > or = 200 mg/dL on at least 2 occasions.

2. Very high glucose levels (>450 mg/dL in adults) may result in Diabetic Ketoacidosis & is considered critical.

Test Name	Result	Unit	Bio. Ref. Interval	Method
GLUCOSE, POST PRANDIAL (PP), 2 HOURS , *SODIUM FLUORIDE PLASMA (2 HR)*	120	mg/dL	Non-diabetic <140 ~ Impaired glucose Tolerance 140 - 200 ~ Diabetic >200	Hexokinase

Comment:

It is recommended that FBS and PPBS should be interpreted with respect to their Biological reference ranges and not with each other.

Conditions which may lead to lower postprandial glucose levels as compared to fasting glucose levels may be due to reactive hypoglycemia, dietary meal content, duration or timing of sampling after food digestion and absorption, medications such as insulin preparations, sulfonylureas, amylin analogues, or conditions such as overproduction of insulin.

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Consultant Pathologist

SIN No:704638167PP

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DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval	Method
HBA1C (GLYCATED HEMOGLOBIN) , *WHOLE BLOOD EDTA*				
HBA1C, GLYCATED HEMOGLOBIN	**6.4**	%		HPLC
ESTIMATED AVERAGE GLUCOSE (eAG)	137	mg/dL		Calculated

Comment:

Reference Range as per American Diabetes Association (ADA) 2023 Guidelines:

Reference Group	**HbA1c (%)**	**HbA1c (mmol/mol)**
Non-Diabetic	<5.7	<38.8
Prediabetes	5.7 – 6.4	38.8 – 46.5
Diabetes	≥ 6.5	≥47.5
Diabetics		
Excellent control	6 – 7	42.1 – 53.0
Fair to good control	7 – 8	53.0 – 63.9
Unsatisfactory control	8 – 10	63.9 – 85.8
Poor control	>10	>85.8
Note: HbA1c IFCC (mmol/mol) = (10.93 x HbA1c NGSP (%)) – 23.50		

1. HbA1C is recommended by American Diabetes Association for Diagnosing Diabetes and monitoring Glycemic Control by American Diabetes Association guidelines 2023.
2. Trends in HbA1c values is a better indicator of Glycemic control than a single test.
3. Low HbA1c in Non-Diabetic patients are associated with Anemia (Iron Deficiency/Hemolytic), Liver Disorders, Chronic Kidney Disease. Clinical Correlation is advised in interpretation of low Values.
4. Falsely low HbA1c (below 4%) may be observed in patients with clinical conditions that shorten erythrocyte life span or decrease mean erythrocyte age. HbA1c may not accurately reflect glycemic control when clinical conditions that affect erythrocyte survival are present.
5. In cases of Interference from Haemoglobin variants (HbF >25%, Homozygous Hemoglobinopathies) in HbA1C testing, alternative methods (Fructosamine) estimation is recommended for Glycemic Control. Abnormal Haemoglobin studies (HPLC/Electrophoresis) is recommended for detection of Hemoglobinopathies.

Dr.Mahalakshmi S
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Consultant Pathologist

SIN No:704638167E

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DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval	Method
LIPID PROFILE , *SERUM*				
TOTAL CHOLESTEROL	153	mg/dL	< 200	CHOD-PAD
TRIGLYCERIDES	**157**	mg/dL	< 150	GPO-PAP
HDL CHOLESTEROL	**37**	mg/dL	>=40 Desirable	Enzymatic Immunoinhibition
NON-HDL CHOLESTEROL	116	mg/dL	<130	Calculated
LDL CHOLESTEROL	84.6	mg/dL	<100	Calculated (Friedewald)
VLDL CHOLESTEROL	**31.4**	mg/dL	<30	Calculated
CHOL / HDL RATIO	4.14		0-4.97	Calculated
ATHEROGENIC INDEX (AIP)	**0.27**		<0.11	Calculated

Comment:

Reference Interval as per National Cholesterol Education Program (NCEP) Adult Treatment Panel III Report.
Below Table as per Lipid Association of India (LAI) (2023) and Cardiological Society of India (CSI) (2024) Guidelines and Consensus Statements for Dyslipidemia Management:

	Low Risk	Moderate Risk	High Risk	Very High Risk	Extremely High Risk
Total Cholesterol	< 200	< 200	200 – 239	≥ 240	≥ 240
Triglycerides	<150	< 150	150 – 199	200 – 499	≥ 500
LDL	< 100	100 – 129	130 – 159 Target for High Risk: < 70	160 – 189 Target for Very High Risk: < 50	≥ 190 Target for Extreme Risk – Category A,B :< 30,Category C: 10 – 15
HDL	≥ 60	≥ 60	M: <40, F: <50	<30	<30
Non-HDL Cholesterol	< 130	130 – 159	160-189 Target for High Risk: < 100	190 – 219 Target for Very High Risk: < 80	≥220 Target for Extreme Risk – Category A,B :< 60, Category C: 40 – 45

Note: **Low risk** – No known risk factor of cardiovascular disease. **Moderate risk** – Any one risk factor, eg; smoking/hypertension/diabetes mellitus etc. **High risk** – Two or more risk factors without any disease manifestation, chronic kidney disease, long-standing diabetes mellitus existing for >10 years, history of heterozygous familial hypercholesterolemia.
Very high risk – clinical evidence of coronary artery disease, long-standing diabetes mellitus existing for >20 years, etc. **Extremely high risk** – recurrent vascular events.
(Category A- CAD with >1 feature of High risk group, Category B- CAD with >1 feature of very high risk group or poly-vascular disease, Category C- Recurrent CAD with other significant risk factors as high lipoprotein a, extensive coronary calcium)

1. Measurements for Lipids (Especially Triglycerides) can show physiological (Dependent on diet,10-12 hrs fasting pre-test condition) & analytical variations.
2. Lipid Association of India (LAI) recommends screening of all adults (>20 yrs) for Atherosclerotic Cardiovascular Disease (ASCVD) risk factors with lipid profile testing. The association recommends testing also to include Apolipoprotein B & Lipoprotein (a) for stratification and defining LDL – C targets.

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Consultant Pathologist

SIN No:704638167S

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DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval	Method
LIVER FUNCTION TEST (LFT) WITH GGT , *SERUM*				
BILIRUBIN, TOTAL	0.35	mg/dL	0-1.2	Diazo
BILIRUBIN CONJUGATED (DIRECT)	0.12	mg/dL	0-0.3	Diazo
BILIRUBIN (INDIRECT)	0.23	mg/dL	0.0-1.1	Calculated
ALANINE AMINOTRANSFERASE (ALT/SGPT)	18	U/L	10-35	IFCC with Pyridoxal Phosphate
ASPARTATE AMINOTRANSFERASE (AST/SGOT)	**9.0**	U/L	10-50	IFCC with Pyridoxal Phosphate
AST (SGOT) / ALT (SGPT) RATIO (DE RITIS)	0.5		<1.15	Calculated
ALKALINE PHOSPHATASE	85.33	U/L	35-104	IFCC
PROTEIN, TOTAL	7.78	g/dL	6.4-8.3	Biuret
ALBUMIN	4.17	g/dL	3.5-5.2	Bromo Cresol Green
GLOBULIN	**3.61**	g/dL	2.0-3.5	Calculated
A/G RATIO	1.16		0.9-2.0	Calculated
GAMMA GLUTAMYL TRANSPEPTIDASE (GGT)	20.00	U/L	6-42	IFCC

Comment:

LFT results reflect different aspects of the health of the liver, i.e., hepatocyte integrity (AST & ALT), synthesis and secretion of bile (Bilirubin, ALP), cholestasis (ALP, GGT), protein synthesis (Albumin) Common patterns seen:

1. Hepatocellular Injury: *AST – Elevated levels can be seen. However, it is not specific to liver and can be raised in cardiac and skeletal injuries.*ALT – Elevated levels indicate hepatocellular damage. It is considered to be most specific lab test for hepatocellular injury. Values also correlate well with increasing BMI. Disproportionate increase in AST, ALT compared with ALP. AST: ALT (ratio) – In case of hepatocellular injury AST: ALT > 1In Alcoholic Liver Disease AST: ALT usually >2. This ratio is also seen to be increased in NAFLD, Wilsons's diseases, Cirrhosis, but the increase is usually not >2.Note- If both SGPT and SGOT are within reference range then AST:ALT (De Ritis ratio) does not have any clinical significance.
2. Cholestatic Pattern:*ALP – Disproportionate increase in ALP compared with AST, ALT. ALP elevation also seen in pregnancy, impacted by age and sex.*Bilirubin (Direct) and GGT elevated- helps to establish hepatic origin.
3. Synthetic function impairment:*Albumin- Liver disease reduces albumin levels, Correlation with PT (Prothrombin Time) helps.
4. Associated tests for assessment of liver fibrosis - Fibrosis-4 and APRI Index.

Dr.Mahalakshmi S
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Consultant Pathologist

SIN No:704638167S

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DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval	Method
RENAL PROFILE/KIDNEY FUNCTION TEST (RFT/KFT) , *SERUM*				
CREATININE	0.61	mg/dL	0.5-0.9	Jaffe
.eGFR - ESTIMATED GLOMERULAR FILTRATION RATE	114.40	mL/min/1.73m²	>60	CKD-EPI Formula 2021
UREA	23.45	mg/dL	13-43	Urease
BLOOD UREA NITROGEN	11.0	mg/dL	8.0 - 23.0	Calculated
URIC ACID	3.64	mg/dL	2.5-6	Uricase
CALCIUM	9.24	mg/dL	8.6-10	NM-Bapta
PHOSPHORUS, INORGANIC	**4.72**	mg/dL	2.5-4.5	Phosphomolybdate Complex
SODIUM	136	mmol/L	136-145	ISE (Indirect)
POTASSIUM	4.8	mmol/L	3.5-5.1	ISE (Indirect)
CHLORIDE	98.5	mmol/L	98-107	ISE (Indirect)
PROTEIN, TOTAL	7.78	g/dL	6.4-8.3	Biuret
ALBUMIN	4.17	g/dL	3.5-5.2	Bromo Cresol Green
GLOBULIN	**3.61**	g/dL	2.0-3.5	Calculated
A/G RATIO	1.16		0.9-2.0	Calculated

Dr.Mahalakshmi S
M.B.B.S,M.D(Pathology)
Consultant Pathologist

SIN No:704638167S

Patient Name	: Mrs. SRIPRIYA	Collected	: 08/Feb/2026 09:04AM
Age/Gender	: 42 Y 0 M 0 D /F	Received	: 08/Feb/2026 12:58PM
UHID/MR No	: DRUL.0000019139	Reported	: 08/Feb/2026 02:06PM
Visit ID	: DRULOPV19672	Status	: Final Report
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DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval	Method
HOMOCYSTEINE , *EDTA PLASMA*	9.54	µmol/L	<15	Enzymatic

Comment:

Elevations in plasma homocysteine may be the result of deranged Vit B12 metabolism, some chronic medical conditions such as chronic renal and hepatic insufficiency, post menopause, smoking, Homocystinuria and certain drugs. Elevated levels of homocysteine may be used to exclude or confirm deficiencies of vitamin B12 or folate.

Elevations in homocysteine levels have been used as an independent risk factor for Thromboembolic episodes in coronary or cerebral vascular disease, especially in young patients.

Dr.Devi R.S
PhD(Biochemistry)
Consultant Biochemist

SIN No:BI30446711

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DEPARTMENT OF IMMUNOLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
TSH (Ultrasensitive/4thGen) , *SERUM*	**5.450**	µIU/mL	0.270-4.20	ECLIA

Comment:

TSH is a glycoprotein hormone secreted by the anterior pituitary. TSH is a labile hormone & is secreted in a pulsatile manner throughout the day and is subject to several non-thyroidal pituitary influences. Significant variations in TSH can occur with circadian rhythm, hormonal status, stress, sleep deprivation, caloric intake, medication & circulating antibodies.

It is important to confirm any TSH abnormality in a fresh specimen drawn after ~ 3 weeks before assigning a diagnosis, as the cause of an isolated TSH abnormality.

For Pregnant Women	Bio Ref Range for TSH in µIU/mL
First trimester	0.33 – 4.59
Second trimester	0.35 – 4.10
Third trimester	0.21 – 3.15

Dr.Mahalakshmi S
M.B.B.S,M.D(Pathology)
Consultant Pathologist

SIN No:704638167S

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DEPARTMENT OF CLINICAL PATHOLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
COMPLETE URINE EXAMINATION (CUE) , *URINE*				
Physical Examination				
COLOUR	PALE YELLOW		PALE YELLOW	Scattering of light
TRANSPARENCY	CLEAR		CLEAR	Scattering of light
pH	5.50		5-7.5	Bromothymol Blue
SP. GRAVITY	1.030		1.002-1.030	Bromothymol Blue
BIOCHEMICAL EXAMINATION				
URINE PROTEIN	NEGATIVE		NEGATIVE	Protein Error of Indicator
GLUCOSE	POSITIVE (+)		NEGATIVE	GOD-POD
URINE BILIRUBIN	NEGATIVE		NEGATIVE	Diazonium Salt
URINE KETONES (RANDOM)	NEGATIVE		NEGATIVE	Sodium nitro prusside
UROBILINOGEN	NORMAL		NORMAL (0.1-1.8mg/dl)	Diazonium salt
NITRITE	NEGATIVE		NEGATIVE	Sulfanilic acid
LEUCOCYTE ESTERASE	NEGATIVE		NEGATIVE	Diazonium salt
CENTRIFUGED SEDIMENT WET MOUNT AND MICROSCOPY				
Pus Cells	1-2	/hpf	0-5	Microscopy
EPITHELIAL CELLS	2-4	/hpf	<10	Microscopy
RBC	NIL	/hpf	0-2	Microscopy
CASTS	NIL		0-2 Hyaline Cast	Microscopy
CRYSTALS	ABSENT		ABSENT	Microscopy
OTHERS	NIL			Microscopy

Comment:

All urine samples are checked for adequacy and suitability before examination. All abnormal chemical examination are rechecked and verified by manual methods. Microscopy findings are reported as an average of 10 high power fields.

***** End Of Report *****

Dr.Mahalakshmi S
M.B.B.S,M.D(Pathology)
Consultant Pathologist

SIN No:704638167U

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10. This report is not valid for medico legal purposes. It is performed to facilitate medical diagnosis only

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